The Execution Sequence: Why Order Is Not Optional

The six pillars of healthcare transformation — Policy, Technology, Economics, Clinical, Equity, and Operations — are often presented as a framework of simultaneous imperatives. Each pillar is necessary. Each depends on the others. Remove any one and the system fails. This is true. But it obscures a critical practical question that every health system leader, state official, and reform architect must answer before a single dollar is spent or a single care model is redesigned: in what order do you actually build this?

The answer is not arbitrary, and it is not the order in which the pillars are typically defined or discussed. The execution sequence is determined by dependency logic — the hard reality that some things cannot function until other things are already in place. Getting the order wrong does not merely slow progress; it produces structural failures that are expensive to reverse and politically difficult to explain.

Vermont provides both the positive and negative proof cases for this argument. The Blueprint for Health and the AHEAD Model demonstrate what is achievable when the foundational stages are built in the right order. OneCare Vermont, the state's Accountable Care Organization, demonstrates with equal clarity what happens when foundational stages are skipped or assumed. Most instructively, OneCare's failure validates the specific sequencing argument this chapter makes: that Technology must precede Economics-as-management, not follow it. Understanding why leads directly to the correct execution order.

The OneCare Failure: A Sequencing Autopsy

Before presenting the execution sequence, it is worth examining OneCare Vermont's failure in structural terms, because the failure was not random and it was not primarily the result of bad intentions or poor execution. OneCare failed because of sequencing errors baked into its design from the beginning. Identifying those errors precisely is more useful than simply noting that the ACO wound down after a decade of operation.

To understand the failure, readers new to this topic need a brief orientation. An Accountable Care Organization, or ACO, is a group of hospitals, physicians, and other providers who coordinate care for a defined patient population with the goal of reducing costs. When the ACO succeeds in keeping patients healthier and reducing unnecessary utilization, it shares in the savings generated for the payer. Vermont created OneCare as its primary ACO vehicle under the Total Cost of Care model — a payment structure that held OneCare financially responsible for the total healthcare spending of an attributed Vermont population. On paper, this was sound design. In practice, it encountered three sequencing failures that compounded into a cascade the organization could not survive.

Sequencing Failure 1: Economics Without Policy Readiness

The most fundamental problem with OneCare was that Vermont attempted to operate a sophisticated payment reform model — one requiring system-wide accountability —

without the policy foundation to enforce participation. Provider participation in OneCare was voluntary. Hospitals, physician practices, and other providers could choose whether or not to join.

This matters structurally, not just administratively. The Total Cost of Care model holds the ACO financially accountable for the total spending of an attributed population. But when high-cost, high-revenue providers remain outside the ACO — because participation is optional — the ACO is held responsible for costs it cannot influence. A hospital system generating significant revenue from inpatient admissions has a rational financial incentive to stay outside a model that rewards reducing those admissions. When it does, the ACO's ability to manage total cost is structurally undermined from day one.

The policy foundation — the legislative and regulatory authority to require participation, or to make opting out economically irrational — was never established for OneCare. Vermont's more recent legislative progression, from Act 167 through Act 51 to Act 68, reflects a hard-won recognition of this failure: mandatory architecture converts voluntary reform into binding structural change. The AHEAD Model's federal agreement builds on this by embedding participation requirements that OneCare never had.

> **What optional participation actually means for a hospital CFO**
>
> *Imagine you are the chief financial officer of a Vermont hospital. Your hospital generates $40 million annually from inpatient admissions. OneCare Vermont is asking you to join an ACO that will hold you financially accountable for reducing those admissions — reducing your revenue. Participation is voluntary.*
>
> *You have three choices: join and accept the risk; join and manage costs without affecting your revenue base; or don't join. For many hospital systems, the rational choice was the third. This is not a failure of values — it is a predictable response to an incentive structure that the policy framework failed to close.*
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> *Vermont's hospital fiscal crisis makes this more urgent: nine of fourteen hospitals reported operating losses in 2023, with projections indicating thirteen of fourteen will be in deficit by 2028. Average silver exchange plan premiums rose from $456 in 2018 to $948 in 2024 — a 108% increase — while median household income grew only 22%. This structural crisis is precisely what Acts 167, 51, and 68 were designed to address by converting voluntary participation into a mandatory statutory requirement.*

Sequencing Failure 2: Economics Without Technology — The 'Managing Blind' Failure Mode

The second and most instructive sequencing failure compounds the first. OneCare attempted to manage a complex, population-level payment model — one involving real financial risk — without the technology infrastructure required to do so. Specifically, OneCare did not build or acquire an integrated data system combining timely clinical data with claims data in a way that made population health management operationally feasible.

For readers new to this topic: claims data is the financial record of healthcare — every bill submitted to an insurer. It tells you what was done, where, when, and at what cost. Clinical data is the medical record — diagnoses, lab results, medications, care plans. It tells you why something was done and what the patient's health status actually is. Neither dataset is sufficient alone for managing a value-based payment model. You need both, integrated, and available in near-real time.

OneCare operated without this integrated infrastructure at meaningful scale. The consequences were severe and entirely predictable. Care managers could not identify high-risk patients proactively — they were working from claims data arriving 60 to 90 days after care was delivered, meaning they were identifying patients who had already been hospitalized rather than preventing hospitalization in the first place. This is the 'Managing Blind' failure mode: assuming downside financial risk under a Total Cost of Care model without the analytics to track total cost of care in real time. The result is unexpected and unexplainable financial losses — not because the model is wrong, but because the organization cannot see what it is supposed to be managing.

This failure directly informs the execution sequence this chapter recommends. OneCare followed what sounds like a logical order: design the payment model, then figure out the technology. But OneCare's experience reveals the flaw in that logic: the gap between 'design the payment model' and 'build the technology to manage it' is precisely where organizations assume risk before they have visibility. The correct sequence closes that gap by requiring the technology substrate to be operational before the payment model goes live.

> **Why timely data matters as much as integrated data**
>
> *Consider a care manager responsible for 500 high-risk patients under a Total Cost of Care model. Her job is to identify patients who are deteriorating and intervene before they are hospitalized.*
>
> *If her data arrives 60 days after the fact, she learns in March that a patient was hospitalized in January. The intervention opportunity is gone. The cost has already been incurred. She is reviewing history, not managing risk.*
>
> *If her data arrives within 24 to 48 hours — integrated clinical and claims feeds updated daily — she can see that a patient filled a new prescription last week, had an abnormal lab result three days ago, and has not seen their primary care provider in six months. She can call today, before the hospitalization happens.*
>
> *Vermont's technology infrastructure for the AHEAD Model addresses this directly: VHCURES provides longitudinal population data; VITL/VHIE provides the clinical data exchange; and the AHS-GMCB Analytics Vendor provides the modeling layer for global budget management and equity monitoring. This is the substrate OneCare never built.*

The Failure Cascade: How Sequencing Errors Compound

The two sequencing failures above did not operate independently. They compounded into a cascade that progressively undermined OneCare's ability to function. Because participation was optional, OneCare could not achieve the provider network coverage required to manage total cost of care. Because it could not manage total cost of care, it could not demonstrate savings. Because it could not demonstrate savings, it could not attract the capital required to build the technology infrastructure that might have allowed it to manage costs more effectively. Because it lacked that technology, its care management remained limited, its financial reporting remained inadequate, and its clinical value remained undemonstrated. And because it could not demonstrate clinical value, skeptical providers had no reason to join — returning the cascade to its starting point.

This is what a sequencing failure looks like at the system level: not a single catastrophic event, but a slow structural unraveling in which each missing foundation makes the next stage harder to execute. OneCare Vermont wound down its ACO operations after roughly a decade. The failure was architectural, not operational.

Resolving the Chicken-and-Egg: Why Technology Precedes Economics

The OneCare failure resolves a tension that has confused reform architects since the beginning of value-based care: the apparent chicken-and-egg relationship between Technology and Economics.

The confusion runs in both directions. On one hand, **you cannot manage economics without technology.** A global budget model without integrated, timely data is a financial accountability structure with no operational mechanism for accountability. On the other hand, **you cannot fund technology without economics.** The infrastructure required for population health management requires significant sustained investment. Someone must decide to fund it.

The resolution requires distinguishing three things that reform frameworks typically conflate:

> Economics-as-design — the analytical work of defining the payment architecture: units of accountability, metrics, risk allocation, performance periods. This is planning work that can and should happen during the technology build. It does not require the technology to be running. A technology vendor building a care management platform needs to know what the payment model is measuring — the design specifies the build requirements.

> Policy-as-capital — the funding of the technology build itself. This is a critical insight that most frameworks miss entirely: the capital for Stage 2 technology infrastructure is a Policy-pillar function, not an Economics-pillar function. Vermont's $195 million Rural Health Transformation award is the clearest example — federal grant capital secured through the Policy pillar that funds the technology substrate. This is fundamentally different from the incentive architecture the Economics pillar creates.

> Economics-as-management — the ongoing operational work of running global budgets: monitoring performance, calculating shared savings, adjusting payments, holding providers accountable. This work absolutely requires the technology infrastructure to already be functional. It cannot begin until the data substrate is in place.

The sequencing logic that follows is unambiguous: Policy secures both the mandate and the capital. Technology gets built, funded by Policy capital, while payment architecture is designed in parallel. Economics goes live — as management, not just design — only when the technology can support it. OneCare inverted this by going live with financial risk before the technology was ready. The result was the 'Managing Blind' failure mode described above.

There is also a second important distinction embedded in the Economics pillar that the new sequence makes clearer: the difference between Incentive Architecture and Transformation Capital. The Economics pillar's structural role is to create the financial environment that makes population health management rational — global budgets, reference-based pricing targeted at 200% of Medicare or below, the 'Balloon Squeeze' logic ensuring that price controls do not simply shift costs into volume expansion. This is permanent structural change to the payment environment. Transformation Capital — the RHT award, the AHEAD EAST Fund providing up to $150 million annually — is time-limited bridge funding for infrastructure. Confusing the two leads to treating one-time grants as permanent subsidies, which is as structurally unsound as treating incentive architecture as capital investment.

The Six Stages of Execution

With OneCare's failure as the empirical anchor and the Technology-Economics dependency resolved, the correct execution sequence can be stated with both logical clarity and practical grounding.

Stage 1: Policy — Mandatory Architecture and Transformation Capital

Policy is the absolute precondition for everything that follows, but its role is larger than simply establishing regulatory permission. The Policy pillar has two functions that no other pillar can perform: it creates the mandatory architecture that prevents high-cost actors from opting out of reform, and it secures the transformation capital that funds the technology build.

Vermont's legislative progression illustrates both functions. Act 167 established the diagnostic foundation — the evidence base demonstrating Vermont's healthcare system had reached a structural breaking point, with nine of fourteen hospitals in operating losses and a cumulative five-year system deficit projected at between $700 million and $2.4 billion. Act 51 moved to the planning phase, defining the transformation architecture. Act 68 operationalized the mandate — converting voluntary reform into binding statutory requirements and establishing the regulatory framework for global budgets and reference-based pricing. The AHEAD Model federal agreement layered in the federal partnership and funding commitments that made the technology build financially feasible.

The $195 million Rural Health Transformation award is a Policy-pillar outcome, not an Economics-pillar outcome. This distinction matters because it clarifies where transformation capital comes from: not from the payment model itself, but from the policy and regulatory relationships that secure federal investment. Reform architects who expect the payment model to self-fund the technology build will consistently find themselves in the same position as OneCare — financially exposed before the infrastructure is ready.

Policy at Stage 1 must address the participation question directly. A reform architecture that allows high-cost, high-revenue providers to opt out of accountability creates a structural gap that no subsequent stage can fill. Vermont's hard experience with OneCare is the proof case. The mandatory architecture of Acts 51 and 68 is the corrective response.

Stage 2: Technology — Building the Substrate Before Assuming Risk

Technology is the operating system on which every subsequent stage runs. This is the sequencing insight that OneCare's failure makes most vividly clear. Technology is not a layer added on top of a functioning reform — it is the foundation that makes reform functional. The entire logic of placing Technology before Economics rests on this point: you cannot assume financial risk under a global budget model before you can see what you are managing.

The technology infrastructure required for serious value-based transformation has several non-negotiable components. First, it must integrate clinical data with claims data. Vermont's infrastructure addresses this through VHCURES — the all-payer claims database providing foundational longitudinal data for population health analytics and total cost of care modeling — and VITL/VHIE, the Vermont Health Information Exchange providing the clinical data substrate for real-time information sharing across the provider network. Neither alone is sufficient. VHCURES without clinical data tells you what things cost but not how to change them. Clinical data without VHCURES tells you about individual patients but not about population-level cost patterns.

Second, the data must be timely. Data arriving 60 to 90 days after care supports retrospective reporting but not proactive care management. The ability to identify patients at risk before they become expensive requires data updated within days. Vermont's AHS-GMCB Analytics Vendor addresses this by providing the modeling needed for hospital global budget design, total cost of care tracking, and equity monitoring — the analytics gap identified in the November 2025 AHS report as a critical missing piece.

Third, the Vermont Clinically Integrated Network functions as a technology-led institution providing shared analytical capabilities to rural hospitals that individually lack the scale to build these functions. Rural transformation cannot depend on every small hospital independently developing sophisticated population health analytics. The shared infrastructure model is both economically rational and operationally necessary.

The payment architecture design happens in parallel during Stage 2 — this is the Economics-as-design work that specifies what the technology needs to do. But the Economics pillar does not go live, does not assume risk, and does not begin performance management until the technology substrate is functional. That is the sequencing discipline that OneCare lacked and that the AHEAD Model is designed to enforce.

Stage 3: Economics — Incentive Architecture on a Visible System

Economics is the third stage, not the second, because its management function requires the technology substrate already to be operational. When the data infrastructure is in place, the Economics pillar deploys the payment architecture that converts population health management from a clinical aspiration into a financially rational imperative.

Vermont's economic architecture has two structural components that work together. Reference-based pricing, targeted at 200% of Medicare rates or below, addresses the unit price problem: Vermont's commercial insurance rates have run far above Medicare, creating a cross-subsidy that is neither sustainable nor equitable. But price controls alone create a 'Balloon Squeeze' — if unit prices are capped without controlling volume, providers simply generate more volume to maintain revenue. Global hospital budgets address the volume dimension by setting a fixed total revenue envelope for each hospital, making volume expansion financially irrational. Together, reference-based pricing and global budgets break the fee-for-service trap that has made population health management economically punishing for providers.

The AHEAD EAST Fund — providing up to $150 million annually in Medicare funds — operates alongside the incentive architecture as transformation capital for community health investment. This is the distinction between Incentive Architecture and Transformation Capital in practice: global budgets and reference-based pricing permanently restructure the payment environment; the EAST Fund provides time-limited bridge capital for the workforce development, telehealth infrastructure, and community health worker programs the transformed system requires.

The equity constraint on payment design also becomes operationally urgent at Stage 3. Global budgets and reference-based pricing, if designed without social risk adjustment, can inadvertently penalize safety-net providers serving high-SDOH populations — providers who appear less efficient by standard metrics because their patient populations have more complex social needs. Vermont's Northeast Kingdom is the clearest example: a global budget benchmark calibrated without accounting for the region's underlying social risk profile will generate financial pressure on the region's hospitals that reflects social disadvantage, not operational inefficiency. Social risk adjustment must be embedded in the payment design, not retrofitted afterward.

Stage 4: Clinical — Redesigning Care on Aligned Incentives

Clinical redesign is where the economic return on the entire investment is realized. It is the stage at which payment incentives, technology infrastructure, and operational capacity converge to change what actually happens between a clinician and a patient. The Patient-Centered Medical Home transforms primary care from acute episodic treatment into proactive, team-based population management. The Collaborative Care Model integrates behavioral health into primary care. The Certified Community Behavioral Health Clinic expands access for the highest-need populations. Vermont's Blueprint for Health generated a 5.8 to 1 return on investment from its primary care investment — among the most robust ROI findings in the value-based care literature.

Clinical redesign comes fourth because clinical transformation without economic alignment reverts when the incentives revert. This is among the most empirically robust findings from a decade of payment reform research. Clinicians redesign their practice in response to the incentives they face and the data available to them. Without a payment model that rewards the redesigned care — already live at Stage 3 — and without the technology to identify which patients need which interventions — already built at Stage 2 — clinical transformation is isolated, unsustainable, and unable to demonstrate the population-level outcomes that justify its cost.

Stage 5: Equity — Constraining Dependency and System Calibration

Equity is fifth in execution sequence but must be understood as a constraining dependency that runs through every preceding stage — not a final add-on applied to a finished system. The distinction between equity as a design constraint and equity as an audit function is critical for understanding what Stage 5 actually does.

At Stage 3, equity constrains the payment design through social risk adjustment. At Stage 4, equity constrains the clinical redesign through cultural competence requirements, language access mandates, and targeted deployment in underserved areas. Vermont's 11-point primary care access gap between white and BIPOC residents is not closed by a Stage 5 audit — it is closed by Stages 3, 4, and 5 working in sequence: payment design that does not penalize safety-net providers, clinical redesign that deploys Community Health Teams in underserved areas, and Stage 5 audit that measures whether those interventions are actually closing the gap.

What Stage 5 adds is the systematic, population-level measurement of whether the equity commitments embedded in earlier stages have produced equitable outcomes — and the corrective action, informed by real data from a functioning system, when they have not. Vermont's HRSN screening at scale, the EAST Fund for social determinants, and the Rural Emergency Hospital model provide the operational mechanisms for this stage. The REH model — preserving 24/7 emergency access in geographically isolated communities — is an equity intervention that could not be deployed without the operational and clinical infrastructure of Stages 2 through 4 already in place.

Stage 6: Operations — Closing the Administrative Cost Gap

Operations is the final stage and the execution layer that converts all prior-stage investments into organizational reality. Vermont's administrative cost data makes the operational opportunity concrete: Vermont's critical-access and rural hospitals spend $2,730 per discharge on administration, compared to a national benchmark of $1,427. This $1,303 per-discharge gap — across Vermont's hospital system — represents the primary source of the $100 million or more in projected direct savings from shared services and administrative simplification.

Operations is last in execution sequence not because it is least important — administrative cost reduction is one of the most direct levers for financial sustainability — but because operational redesign requires the technology infrastructure, payment incentives, and clinical models of the preceding stages to already be in place. A hospital system that redesigns its administrative operations before its payment model is live will optimize for a fee-for-service world that the reform is designed to replace. The Rural Health Redesign Center methodology — performing financial and operational baselines for each of Vermont's fourteen hospitals, identifying the specific administrative gaps, and designing the shared services architecture — is an operational function that produces durable results only when the payment environment it is optimizing for is already operational.

The Sequence in Practice: Overlap, Iteration, and Political Reality

The six-stage sequence described in this chapter is a logical structure grounded in dependency relationships, not a rigid waterfall that prohibits parallel work. In practice, payment architecture design proceeds in parallel with the technology build. Operational planning begins before the technology is complete. Clinical pilots run before the payment model fully covers the redesigned care. This is normal and expected.

What the sequence provides is a framework for understanding what happens when dependencies are violated — specifically, when a later stage assumes operational risk before an earlier stage has delivered its foundational output. When a technology build is incomplete, the payment model that depends on it should not go live with full financial risk. When a payment model has not yet aligned incentives, clinical transformation initiatives will face reversion pressure. When workforce is insufficient to deliver a clinical model, the model must be scoped to what is actually deliverable.

The most important practical insight from Vermont's experience — positive and negative — is that the sequence failures that produce the most damage are the ones that appear to be minor timing decisions. OneCare did not make a dramatic choice to ignore technology. It made the reasonable-sounding decision to launch the payment model while technology was still being developed. That decision, compounded by the absence of mandatory participation, produced a failure cascade that no amount of subsequent operational excellence could reverse.

The framework presented in this chapter gives reform architects the analytical basis to resist that pressure — to hold the sequence with discipline while managing the political reality that demands visible progress at every stage, and to distinguish between genuine parallel progress and premature sequencing that builds failure into the system from the start.

Sequencing Summary

Stage	Pillar	Key action	Why this sequence	Vermont anchor
1	**Policy**	Establish enforceable authority, mandate participation, and secure transformation capital	Nothing can be authorized, built, or funded without statutory permission. Policy also secures the capital (RHT award) that funds the technology build. Mandatory architecture prevents high-cost actors from opting out.	AHEAD Model federal agreement; Acts 167, 51, and 68; $195M Rural Health Transformation award; mandatory reporting requirements
2	**Technology**	Build integrated data infrastructure — clinical + claims, real-time — before financial risk is assumed	Technology is the operating system. It must be operational before the Economics pillar goes live. Managing global budgets without integrated data is the 'Managing Blind' failure mode that destroyed OneCare.	VHCURES all-payer claims database; VITL/VHIE clinical data exchange; AHS-GMCB Analytics Vendor; care management platforms; AI-assisted documentation
3	**Economics**	Deploy global budgets and reference-based pricing onto a system that can already see itself	Economics-as-management requires the technology substrate to already be operational. Payment design (Economics-as-design) happens in parallel during Stage 2, but the Economics pillar goes live only when data infrastructure is ready.	Global hospital budgets; Reference-Based Pricing at ≤200% Medicare; AHEAD EAST Fund up to $150M annually; Total Cost of Care model activation
4	**Clinical**	Redesign care delivery models and transform clinical practice on aligned incentives	Clinical transformation is where ROI is realized — but only when payment incentives are live and technology can identify which patients need which interventions.	PCMH transformation; Collaborative Care Model (CoCM); CCBHC expansion; Blueprint for Health 5.8:1 ROI from primary care investment

Stage	Pillar	Key action	Why this sequence	Vermont anchor
5	**Equity**	Audit, calibrate, and correct — including social risk adjustment embedded in payment design	Equity is a constraining dependency that must be embedded in Stages 3–4 and audited here against real population data. Without social risk adjustment, global budgets penalize safety-net providers.	HRSN screening at scale; EAST Fund for social determinants; social risk adjustment in global budgets; Rural Emergency Hospital model
6	**Operations**	Close the administrative cost gap and translate strategy into implemented programs	Operations is the execution layer. Vermont's $1,303 per-discharge administrative gap vs. national benchmark is the primary source of projected direct savings from shared services and simplification.	Rural Health Redesign Center (RHRC) methodology; shared administrative services; 14-hospital transformation planning; Community Health Teams

The table above summarizes the revised execution sequence: Policy → Technology → Economics → Clinical → Equity → Operations. The key change from earlier sequencing frameworks is the placement of Technology before Economics. This change is not cosmetic. It reflects the central lesson of OneCare's failure: that assuming financial risk before the data infrastructure is operational is not a sequencing preference but a structural error with predictable consequences.

Vermont's Blueprint for Health succeeded because it built the technology substrate incrementally over years before the full payment accountability of the AHEAD Model was assumed. OneCare failed because it assumed that accountability first. The execution sequence this chapter recommends ensures that every state and health system learning from Vermont's experience avoids that error by design rather than discovering it through failure.